12. THE HEART : SUMMARY AND PRACTICAL IMPLICATIONS

DURATION : 11:02

STUDY SHEET

COURSE SUMMARY

This video explores the importance of the heart in embryonic development, positioning it as the meeting point of vital forces between sperm and egg. It explains how the heart forms within the primitive streak, interacting with key structures such as the neural tube and mesoderm. The concepts of diastole and systole are also discussed, with particular attention to how these movements can be observed and integrated into osteopathic practice. Practitioners will learn to use embryonic fulcrums to revitalise and balance cardiac and cerebral systems, refining their approach through relevant points of support.

THE HEART: SUMMARY AND PRACTICAL IMPLICATIONS

THE HEART: SYMBOLIC MEETING POINT

The **heart** is considered the symbolic meeting point between the **spermatozoon** and the **ovum**. It is here that a **fundamental pattern** is established, manifesting even on a polarity plane.

FIG. — Two-cell embryo

EARLY APPEARANCE AND DEVELOPMENT

The emergence of the heart is particularly interesting at the level of the **primitive streak**.

It is the remnant below S2, the terminal end of the **notochord** and the site of sensation.

We approach it in its facial environment and its developmental movement. It will reintegrate **secondary neurulation** during the closure and establishment of the lesser pelvis.

Secondary neurulation is the space between the **filum terminale** and the **caudal bud**, involving differential growth of the **neural tube** relative to the anterior **endoblast**, with an opening of the lumen.

The anterior endoblast is the embryonic endoblast, but here, at the **mesoblastic** level and also at the heart level.

In this wave with the primitive streak, which ends at S2, a **primitive cardiac force** is already present. The presumed location in the epiblast is part of the heart's potential; it is the earliest identified location.

Cardiogenic cells are found at the epiblast level, in the presumptive layer of the epiblast, even before mesoderm formation.

We will find this presumptive lamina again. A growth movement is followed by congestion, then the integration and meeting of the two tubes, organised from the periphery by the **amniotic cavity**. The neural tube, the cardiac tube, and the integration of the diaphragm contribute to the establishment of the heart.

This occurs at the same time as the closure of the neural tube and the integration of the external **coelom** into the internal coelom.

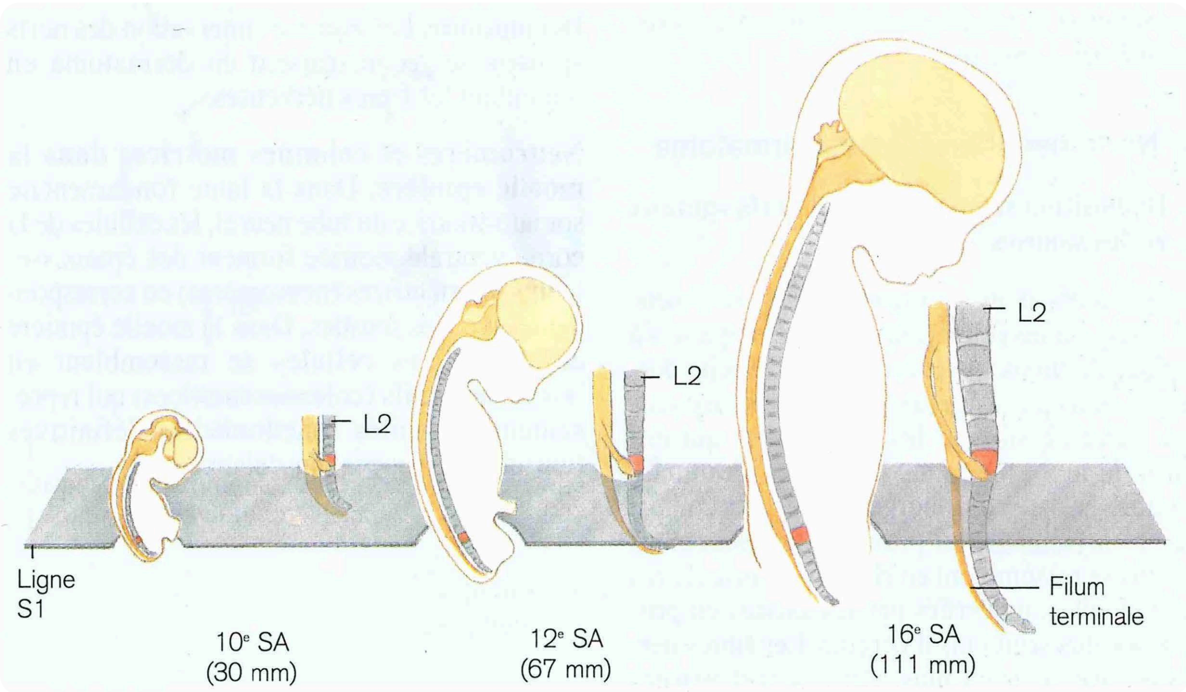

FIG. — Relative growth of spinal cord

THE PRIMITIVE STREAK AND THE CARDIAC FULCRUM

The earliest identifiable location is the primitive streak.

The notochord terminates at S2. The space between S2 and the end of the primitive streak contains the remnant of the primitive streak.

The **embryonic fulcrum** of the heart is on the coccyx.

The heart then adopts a rather **diastolic** posture, seeking energy and volume. It descends in its movement.

The heart also presents a fulcrum, i.e., a point of origin, which it can manifest differently.

CARDIAC MOVEMENTS AND STRUCTURES

A movement is established between the **brain**, with **cephalisation**, and the heart, **cardialisation**, allowing them to meet.

At the same time, the two endocardial tubes meet on the midline. This is followed by the **looping** of the heart, then the establishment of the **cardiac sinusoid** and the **aortic arch**, with the great vessels.

The **caval axis** serves as a landmark.

It is important to observe whether the heart expresses itself on a systolic or diastolic plane. Facially, it seeks more diastole or systole.

Relative to the iliac, it seeks much more **horizontality**, which indicates a diastolic movement. We can even go as far as to seek the **aortic pulse**.

CEREBRAL AND CARDIAC DEVELOPMENT

We will progressively integrate brain development in relation to developmental movement.

The first movement of brain development was **expansion**. This expansion depended on the **longitudinal axis** or the **mesenteric axis**. It depends on vitelline integration, the remnant of which will be found on the mesenteric axis.

Then, another important level for nutrition is the system between the **spleen** and the **liver**, with the return to the heart via the **vena cava**. Then, the aortic information will manifest through four or two entry points: carotid, above, below, vertebral.

We thus integrate brain development with the heart and the mesenteric system. By working on the brain, we observe which field needs to open. We can refine perceptions to determine the areas to work on, for example, on the carotid system in relation to the heart.

This can be done by providing simple points of support, whether they are embryonic fulcrums or points of support in the construction, to revitalise the regenerative force.

A fulcrum is a point of emergence. All the developmental force, such as the integration of the **vitelline vesicle** into an artery (a remnant of the developmental pathway and not an embryonic fulcrum), lies between the fulcrum and the finality.

We are beginning to perceive the game we are about to engage in, as we will be working on specific arteries. The goal is to restore **full homeostasis**, full vitality, full health.

ORIGIN OF THE FULCRUM AND MESODERMAL WAVE

I will touch upon the origin of its fulcrum, which is indirectly found at the coccygeal level, then which will gradually integrate into its system through the developmental movement of the **manotococcus**, this wave with this aspiration at the mesoderm level.

This is information that restructures, revitalises, restores strength to the tissue, restores function.

The development of embryonic forces allows the development of the system's main function.

THE CENTRAL ROLE OF THE DIAPHRAGM

The great master for all this will be the **diaphragm**. It is important to position oneself well on this diaphragm with three levels:

- **Amniotic cavity** on the transverse plane
- **Thoracic**
- **Abdominal**

This integration of the coelom, and at the same time the integration of the brain, the heart, the diaphragm, the posterior part of the peritoneum, to return to the **perineum** and, through this tilting of the liver, restore the complete closure of my linea alba, which will provide all the **urogenital** information.

THE LINEA ALBA AND THE UROGENITAL SYSTEM

This is how it will form. The **linea alba** represents the complete integration of the urogenital plane; it is its final story.

On this linea alba, we can look for concentrations of crystallised time, because it is a line of probability in relation to the **erythrocyte**, the formation of blood cells, thus a future, where the plane of the **copula** is also your future.

At the moment of exit, stretching occurs, and this is how the entire external and internal genital system will integrate on this plane of the anterior linea alba. It ends to rebuild.

On a blood cell plane, **erythrocytosis** or **gonocytosis** advance. At some point, the embryo makes a movement. There is a line of initiation in life. At birth, this line, the linea alba, must be opened, and one must be initiated into life.

EMBRYONIC FULCRUMS AND ARTERIAL SYSTEM

The idea will be to restore the correct **embryonic fulcrums**. The arterial system should be seen as an **octopus**.

Tomorrow, we will detail the arterial system at the facial level, as it is a trophic access point to the brain. We will need to understand the development of the carotid system and the vertebral system.

These two systems, with all the information on the **Circle of Willis** and the anterior, middle, posterior **cerebral arteries**, as well as the sinuses or valves (of Brechet or Thierry Goïdienne, etc.), are spaces of equilibrium.

CEREBRAL VASCULAR BALANCE

We need a balance between vascular supply and venous return. We now know that there is a **lymphatic system** in the brain.

To eliminate surpluses, some may have **lymphatic** or venous **congestion** in the brain. This is observed in people who are a little swollen, a little oedematous. The return is not happening.

A rebalancing work on the venous plane is then necessary, whether at the level of the **superior** or inferior **vena cava**. This is important to avoid congestion, especially in the ears, and anything that cannot be evacuated.

THERAPEUTIC POSTURE

I position myself well at the coccyx and the heart. One can visualise the **angle of Louis**, the vascular trunk of the aortic pulmonary type.

The heart is on the **longitudinal axis**. Only a part of it is dilated to the left, with a developmental phase on its apex to the left.

You will position yourself on the longitudinal plane of the heart. By giving a fulcrum here and a point of support on the heart, my attention is in space, neutral, and we let the system organise itself.